Telehealth Staff Guide

Women's Healthcare Associates

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Patient Services

Scripting for Telehealth visits:

Problem visit / Telehealth helpful scripting

From what you describe, your need would be best met by a Telehealth video appointment with a provider for assessment / evaluation. I have an opening _____ with _____; can I go ahead and schedule you for that?

Great, and I do want to let you know this is a regular visit, meaning your insurance company will be billed. Depending on your benefits, you could be responsible for a copay or deductible and co-insurance requirements.

- **Possible Patient Follow-up Question:** Can you just help me? That's what I normally do when I call
  - **Answer:** Telehealth appointments are our standard for this type of care

- **Possible Patient Follow-up Question:** Does the Telehealth visit cost the same?
  - **Answer:** Virtual visits are billed to your insurance company just like a regular office visit. Depending on your benefits, you may be responsible for a co-pay or deductible and coinsurance requirements

- **Possible Patient Follow-up Question:** Well, if I'm going to be billed, I'd rather be seen in person.
  - **Answer:** A Telehealth visit is the best way for you to be seen for this type of care. If you and your provider decide you need to be seen in person after the Telehealth visit, we will get you scheduled to come in

Problem / Advice triage

I would like to have you speak with one of our advice nurses who can determine how to best meet your need, which may include scheduling you for an in-person or telehealth video visit with one of our providers. Can I go ahead and transfer you?

Note: See Telehealth Guide chapter titled ***What to transfer to Advice over scheduling a Telehealth Visit***

Giving Patient Options

As we continue to work on providing continuity to our patients (being able to see their provider whenever possible) as well as providing patient centered care where patients are given choices of appropriate appointment times, we want to ensure all patient services team members feel comfortable and confident with the following options when scheduling patients:

… "Based on what you're telling me, here are the options I can offer you…"

1. A telehealth appointment today at ***, with one of your provider's partners to discuss your concern. If you end up needing in person testing we can schedule you for a quick in person appointment for testing only

2. A telehealth appointment tomorrow *** with your provider to discuss your concern. If you end up needing in person testing we can schedule you for a quick in person appointment for testing only

3. The next in person appt I have at this clinic is with ***, one of your provider's partners and that appt is on ***.

4. The next in person appt I have with your provider is on *** (which depending on the appointment reason could be too far out)

From there we will work with the patient to determine the best course of action for appointment timing based on their scheduling need. Reach out to the Advice Nurses for any medical questions

Telehealth Key Notes:

- Telehealth visits should be offered only as **video visits** and if the patient has limitations or is unable to do video, then we should get them scheduled for an in person visit (No phone visits). Note, this is different than if a provider is on a video visit and has to switch to phone for various reasons. If you have questions regarding this, ask your site supervisor

- If the patient needs an interpreter for their Telehealth visit, we will utilize Linguava, Roxana Alviar Volesky (WHA's Spanish Internal Interpreter), or a clinician with the ability to interp.

- After scheduling a Telehealth visit, give these instructions to the patient:

"At the time of your visit, you will receive a text message from your provider. All you have to do is click the link in the text message and you will be connected via video with your provider for your visit. A few things to remember: try to be in a quiet, private location with good cell service or reliable Wi-Fi. For best audio quality, we encourage you to use headphones. If you are not able to use head phones, avoid placing your phone on a surface that blocks the speaker/microphone. Verify that your smartphone's software is up to date before logging into your call to avoid video and audio disruptions."

Appointment Note Guide

If **Roxana (WHA's Internal Spanish Interp.)** is the interpreter for the appt, use these Appt Notes*:*

VIDEO Visit: Pt aware text link, Roxana to interpret-3rd party call them at 503-905-4355

How to schedule with Roxana link

Appointment types	Appt Notes
Advice Triage Telehealth	VIDEO Visit: Pt aware Text Link - ADVICE reason -ph# 000-000-0000
• EST Gyn • OB Prim Consult • OB Telehealth • Postpartum • BH	VIDEO Visit: Pt aware of text link - Wks gestation, LMP or Visit reason - ph# 000-000-0000
OB Intake	LMP - Ph# 000-000-0000
New Patient GYN Telehealth	VIDEO Visit: Pt aware of text link and consent forms - Wks gestation, LMP or Visit reason - ph# 000-000-0000
Linguava Interpreter needed for any Telehealth	Linguava needed, see telephone encounter -Visit reason

Day of workflow for Telehealth Visits:

Provider workflow:

- Opens Doximity on their phone
- Sends Text to patient
- Joins Appointment with patient

- If the patient *cannot* be reached via video or telephone call, the provider will send a message to the patient services team to reach out to patient to reschedule

Patient workflow:

- Clicks the link in the received text
- Joins the appointment with their clinician

Clinical Services workflow:

- For Wellness or New Patient Telehealth visits the Medical Assistant will call the patient prior to do the prepping of the chart. The MA will check in the patient for their visit **if all consent forms have been signed by the patient (see Medical Assistant/Rooming Staff chapter for more detail).** If they haven't signed their consent forms and/or the MA cannot get ahold of the patient to do them prior to the prepping of the chart, then the appointment **will need to be rescheduled**

End of the Day reminders for the Patient Services staff

- At the end of each day a designated PSS member for their site or hub will text all the clinicians that saw patients for Telehealth visits that day. They will ask the provider to report all No-Show patients that they had for that day

- That same PSS will then manually No-Show those specific patients off of the DAR or will delegate as needed

- Once that has been completed, the PSS will then delegate a few Patient Service members to check-in all remaining Telehealth visits left on the DAR (**Reminder:** there should **NOT** be any New Patient Telehealth patients seen without electronic consent forms signed prior to visit)

  - The PS team, after the visit or the next day, will **check out the visits that were completed. The PS team** looks to see if the provider had any follow up scheduling instructions for the patient and will send a Staff Message to appropriate pools if follow up is necessary

- If the provider was unable to reach the patient but the front office staff member did get a hold of the patient later that day to reschedule the missed visit, **Reschedule** the Telehealth visit (do NOT No Show the visit)

- Use the appropriate cancellation reason and enter any cancellation comments that would be helpful as to why the appointment was rescheduled.
  - There are many cancellation reasons that could work such as Connectivity Issues, Pending Reschedule, Provider (add comments), Patient (add comments) etc.

Interpreter services for Telehealth visits

Workflow for Bilingual Providers

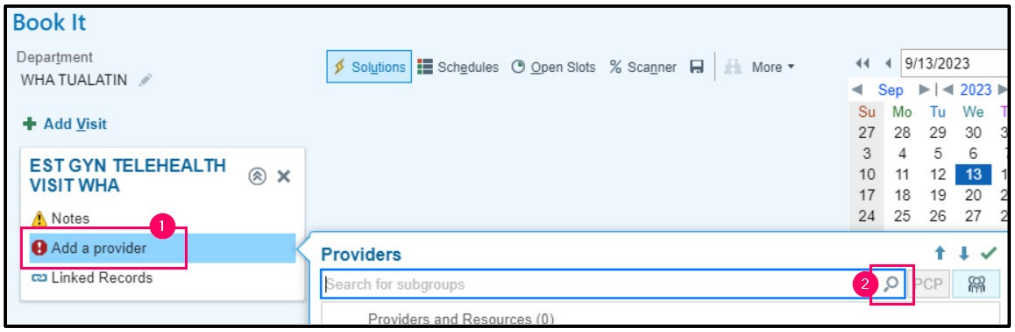
If the patient needs an interpreter for their appointment, **prior to scheduling**, check to see if there is a clinician on the list below available for Telehealth visits for the appropriate language. If you do schedule with one of these clinicians **add in the appointment notes**: *"No interpreter needed/provider fluent"*

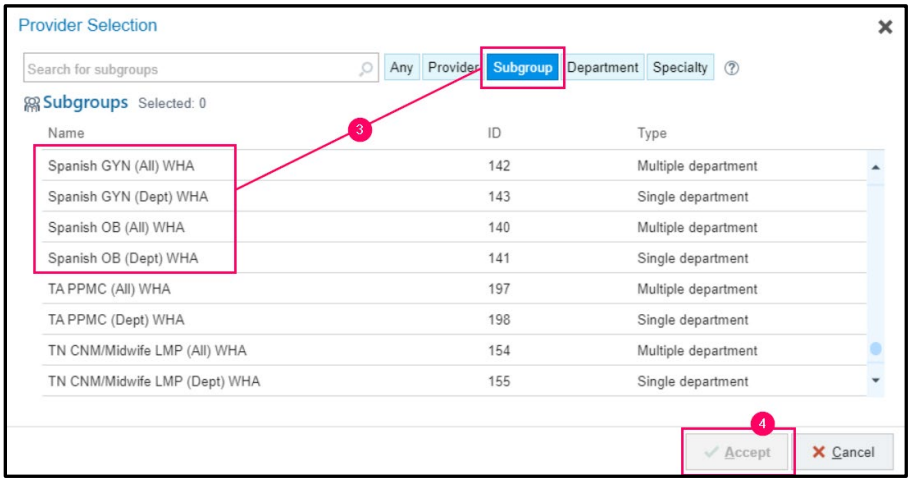
Bilingual Providers at WHA	Location	Language	Able to do Televisits without an outside interpreter
Paige Atkinson, CNM	GW	Spanish	YES
Marisa Bryman, CNM	MBC	Spanish	YES
Liz Labby, CNM	GW	Spanish	YES
Gina Cardona, CNM	GW	Spanish	YES
Jessie Kerstetter, CNM	GW	Spanish	YES
Shirley Ma, CNM	GW	Spanish / Cantonese / Mandarin	YES
Carissa St. Onge Carneiro, CNM	MBC	Portuguese / Spanish	YES
Dr. Hadiashar	NWG TN	Spanish / Farsi	YES
Dr. Lannon	NWP East & West	Spanish / Dutch	YES
Dr. Tronnes	NWP East & West	Spanish	YES
Mary Koski-Vogt, CNM	PKN	Spanish	YES
Jody Lindwall, CNM	PKN	Spanish	YES
Dr. Monnie	PKN	Spanish	YES
Dr. Cabrera	PKN	Spanish	YES
Yvette Rodriguez, CNM	PKN	Spanish	YES
KC Bly, CNM	OC	Spanish	YES
Dr. Bennett	NB	Spanish	YES
Erica Koltenuk, CNM	NB	Spanish	YES
Dr. Geller	NB	Russian / Spanish	YES
Dr. Fowler	TN	Spanish	GYN NO

There are also Subgroups built in EPIC scheduling that allow you to sort via clinicians that speak Spanish *(see image below):*

*If **Roxana** is interp. for the appt, use these Appt Notes:* VIDEO Visit: pt aware text link, Roxana to interpret-3rd party call them at 503-905-4355

Using Spanish subgroups in EPIC while scheduling:





1. Add your Telehealth visit type & click on Add a provider
2. Click on the magnifying glass for options
3. Click on **Subgroup** to see the list. Scroll to the bottom, it will have the **Spanish Subgroups** available to use
4. Click **Accept**

Note: Doing this workflow will automatically change the Interpreter Status to Assigned when a patient has a primary language of Spanish and has marked "Yes" for Needs Interpreter in Registration. Then, when users work the Interpreter Scheduling report they see Assigned and don't need to call interpreter services or do anything.

Interpreter Telehealth visits – Documenting Info

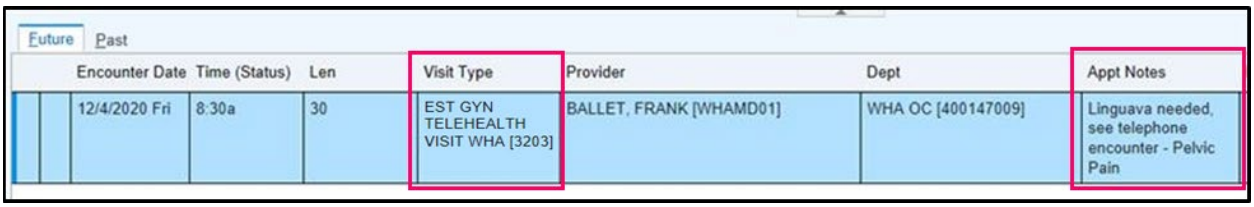
When a patient is scheduled for a Telehealth visit and is needing an interpreter for their appointment, we **only need to do the steps #1-4 below if it is scheduled within 3 business days. The appt. notes do not change regardless of how many days out it is scheduled.**

Appointment types	Appt Notes
Linguava Interpreter needed for any Telehealth *Note if it is a Phone Visit, add that to the beginning of the Appt Notes	Linguava needed, see telephone encounter - Visit reason Or Phone Visit, Linguava needed, see telephone encounter – Visit Reason

A designated Patient Services team member will be continuously running the **Interpreter Scheduling Report** in EPIC for 1, 2 and 3 days out. After running the report, they need to click into each patient's Chart/Chart Review to view their Encounters list to see if another PS team member created the Telehealth Interpreter Documentation encounter already. If you do **not** find one, follow these steps below:

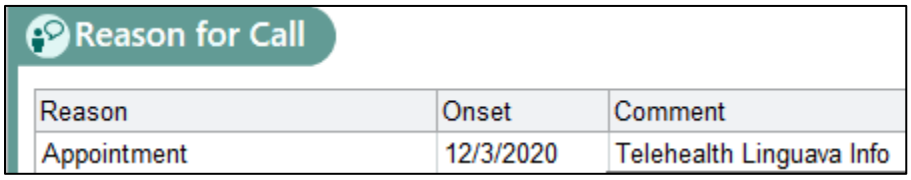
Step #1: Patient is scheduled for Telehealth appointment & interpreter is needed for visit

- Verify appointment notes say = "Linguava needed, see telephone encounter – Visit reason"



Step #2: Initiate Telephone Encounter in patient's Chart

- Reason for Call = "Appointment"
- Comment = "Telehealth Linguava Info"



- Input appropriate smart phrase in Documentation field
  - Video visit = **.WHAVIDEOVISITINTERPRETER**

Step #3: Use F2 to move through highlighted fields and fill in information

.WHAVIDEOVISITINTERPRETER

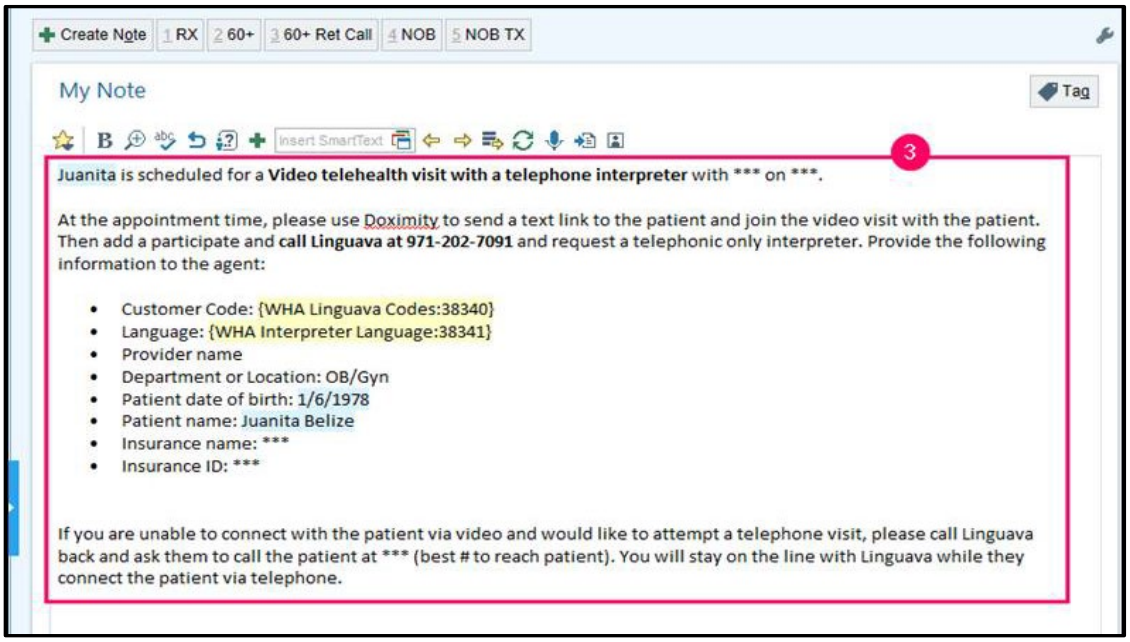
@PREFNAME@ is scheduled for a **Video Telehealth visit with a telephone interpreter** with *** on ***.

At the appointment time, please stat **video** call using Doximity.

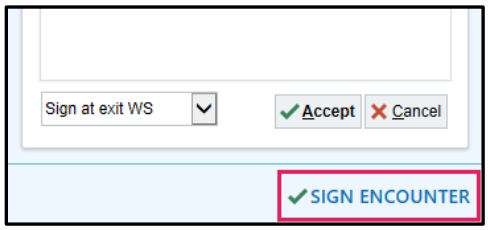
When patient is connected, **call Linguava at 971-202-7091** and request a telephonic only interpreter. Provide the following information to the agent:

- Customer Code: {WHA Linguava Codes:38338}
- Language: {WHA Interpreter Language:38339}
- Provider name
- Department or Location: OB/Gyn
- Patient date of birth: @DOB@
- Patient name: @NAME@
- Insurance name: ***
- Insurance ID: ***

If you are unable to connect with the patient via video and would like to attempt a telephone visit, please call Linguava back and ask them to call the patient at *** (best # to reach patient). Stay on with Linguava while they connect the patient via telephone.



Step #4: **Sign** the Telephone Encounter



Telehealth Interpreter appts scheduled by RN team

When a nurse is scheduling a patient for a telehealth visit who needs an interpreter, a **high priority staff message** will be sent to the corresponding patient services pool for same day appointments. If they schedule an appt within 30 minutes, **they will call** a Patient Services team member asking them to create the Interpreter Telehealth Encounter ASAP.

South Hub Telehealth visit = OC patient services pool

West Hub Telehealth visit = HB patient services pool

East Hub Telehealth visit = TA patient services pool

The nurse will provide a 30 minute lead time when scheduling these appointments, as this will provide adequate time for the patient services team to document the needed interpreter information in the patient's chart for the clinician *(see previous chapter for steps)*.

When a patient services team member receives a high priority staff message indicating a patient needs an interpreter for a same day/upcoming visit they will follow the Interpreter Telehealth Scheduling *(see previous chapter)* instructions to document the needed interpreter information into the patient's chart for the Clinician before the appt.

New Patient Telehealth Appointments

Register per usual (patient should be fully registered prior to scheduling – with the exception of their insurance card scanned– **ok to not have consents signed to schedule but we want to collect this PRIOR to the visit via Myhealth**)

New Patient Telehealth Visit Scheduling tip sheet

- Work with your FC to verify insurance coverage prior to scheduling
- Refer to FC for any Self-Pay patient prior to scheduling

1. Obtain and/or confirm patient E-mail address then sign them up for Myhealth
    - *See chapter below called:* **Pushing E-Consent Forms to the Patient's Myhealth**

2. Schedule using the **NP GYN Telehealth** visit type. Schedule a **Nurse Visit** appt back to back to the TH visit on your assigned Hub's template:
    - West Hub – BR NURSE WHA
    - East Hub – BR NURSE WHA
    - South Hub – BR NURSE WHA
    - NWP – N/A

What? We have to reschedule any new patient telehealth visits that do not answer/call back for chart prep within at least 10 minutes prior to their appointment time **or** who do not complete their consent forms via Myhealth through eCheck-in. You can now push e-consent forms to the patient's Myhealth – see chapter titled ***Pushing E-consent forms to the patient's Myhealth*** for a step by step guide

Why? The provider is not able to complete the visit with the patient if we do not have adequate health history entered in the chart and we cannot bill the patient's insurance if clinic consent forms are not signed.

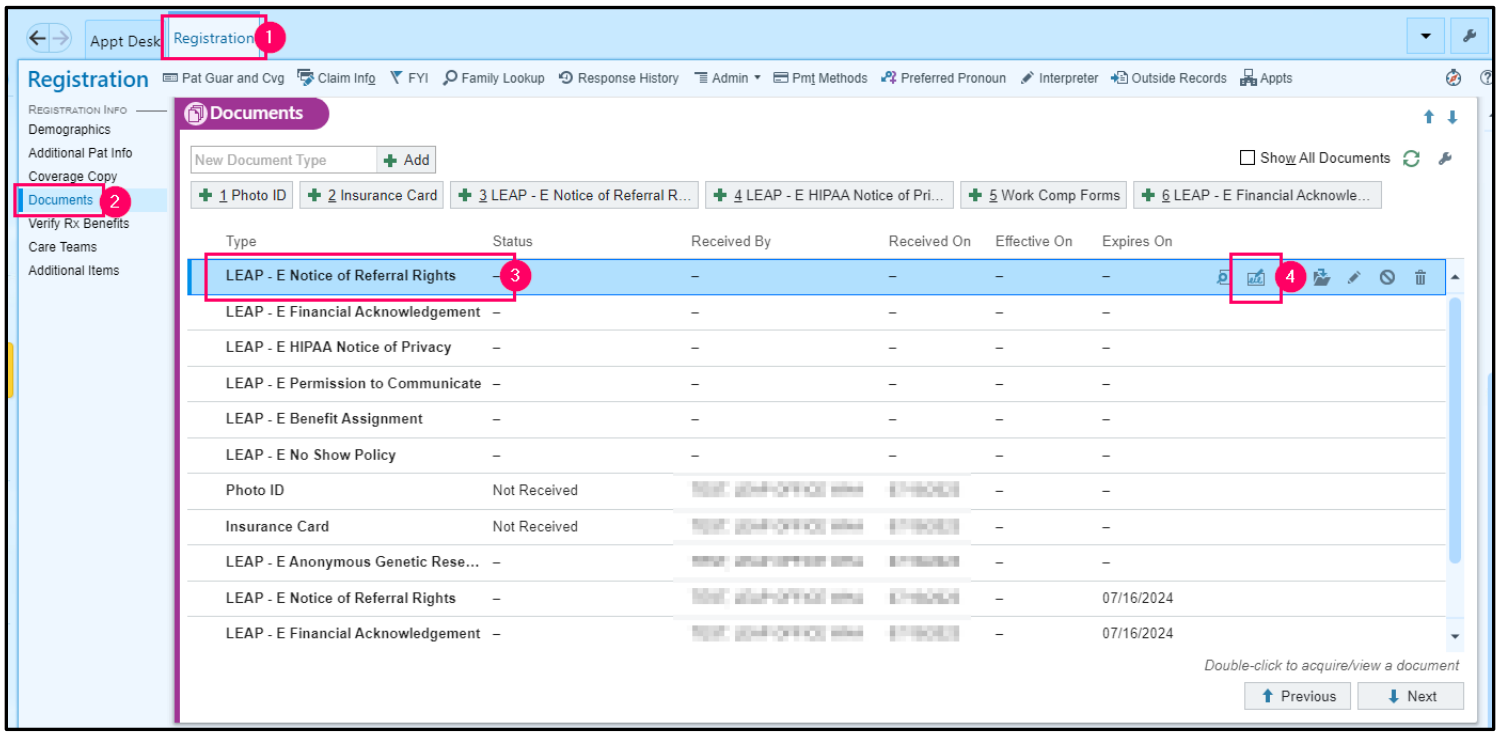
How? Pre Appointment/At time of scheduling: When you schedule a new patient telehealth you will use the scripting *"Please expect a phone call from a medical assistant up to 2 hours prior to your provider visit to discuss your health history, if you are unable to take this call to have your history added to your chart or you do not sign all of the consent forms via eCheck-in (through Myhealth) prior to your visit with your provider we will need to reschedule your visit for a different day or time. This phone call with the medical assistant is the same process we would do to get you roomed to see your provider if you were being seen in person in the office and is an important part of you care with us. Most of our nursing staff are working remotely and the number they call you from will likely not be our office number, please ensure your call blocker is not active during the time you are expecting a call from us and that you answer that possibly unknown number or call us back immediately"*

Patient did not complete chart prep call and/or sign all consent forms prior to visit: Clinical staff member doing nurse visits/chart prep calls sends an URGENT In basket message to PS pool at the provider's designated Telehealth site. This will indicate that the patient did not complete chart prep call in time or did not complete their clinic consent forms via Myhealth and that they will need to be rescheduled.

Pushing E-Consent Forms to New Patient's Myhealth

After scheduling a New Patient for a Telehealth visit, the workflow is to push the electronic consent forms to their Myhealth account immediately after completing the phone call.

These e-sign consents **can be sent prior to the patient having an active MyHealth account,** which means that once the patient creates their Myhealth account the **e-sign consents will be available upon their login.** Also, if the e-sign consents were not signed in advance they would still show up in eCheck-in.
Note: If a patient only signed 3 e-sign consents in advance, the other 3 would still be available in eCheck-in (4 days prior to their visit when it automatically gets pushed to the patient)



1. Go to the patient's **Registration**
2. Click on the **Documents table**
3. Click on the E-Consent form that you would like to send to the patient. You have to send **each form individually** – Here are the e-sig consents that are available to send via MyHealth *(Permission to Communicate is not turned on or included):*

    - Leap – E Anonymous Genetic Research
    - Leap – E Benefit Assignment
    - Leap – E Financial Acknowledgement
    - Leap – E HIPAA Notice of Privacy
    - Leap – E No Show Policy
    - Leap – E Notice of Referral Rights

4. Click **Accept**

Scheduling Same Day New Patient Telehealth Appts

New Patient Same Day Telehealth visits need to be scheduled at **least one hour** before the scheduled appointment time

Do NOT schedule visit if pertinent records are not already in the patient's chart or are in Care Everywhere, unless the visit does not require records (BC consult, options counseling, hasn't been seen for this concern anywhere else, etc.). If you need help looking for records in the patient's chart, contact a PSR.

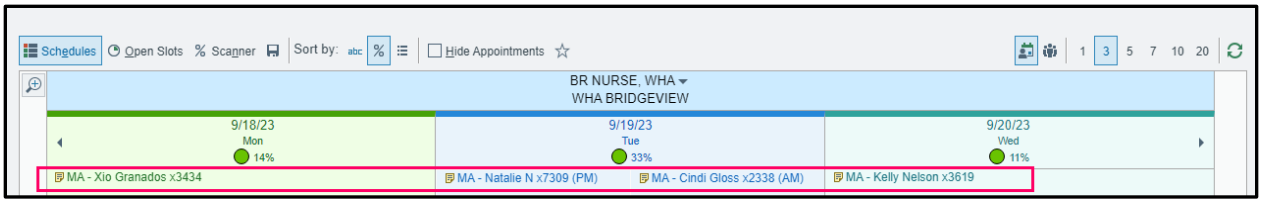
Try to avoid scheduling new patients same day for Irregular bleeding, abnormal bleeding, or post-menopausal bleeding unless they are insistent on being seen today

You can now push e-consent forms to the patient's Myhealth – see chapter titled ***Pushing E-consent forms to the patient's Myhealth*** for a step by step guide

Communication:

Let the patient know that they will receive a phone call from the Clinical team within the 2 hours prior to their visit to prep their chart and if they miss this call OR if they do not sign all the clinic consent forms they will be rescheduled

Next, instant message in Teams the telehealth rooming MA notifying them of the add-on. The rooming MA's name can be found in EPIC at the top of the schedule view of the nurse schedule for each HUB (**BR Nurse** for all hubs)



Postpartum Telehealth Appointments

How to determine if a 3 week PP patient should be in person or Telehealth:

PP3	**Phone/Video** In person or assessed need for in person over video visit: - Hypertension during delivery - 4th degree laceration

Schedule using the **PP Telehealth** visit type. Schedule a **Nurse Visit** appt back to back to the TH visit on your assigned Hub's template:

- West Hub – BR NURSE WHA
- East Hub – BR NURSE WHA
- South Hub – BR NURSE WHA
- NWP – N/A

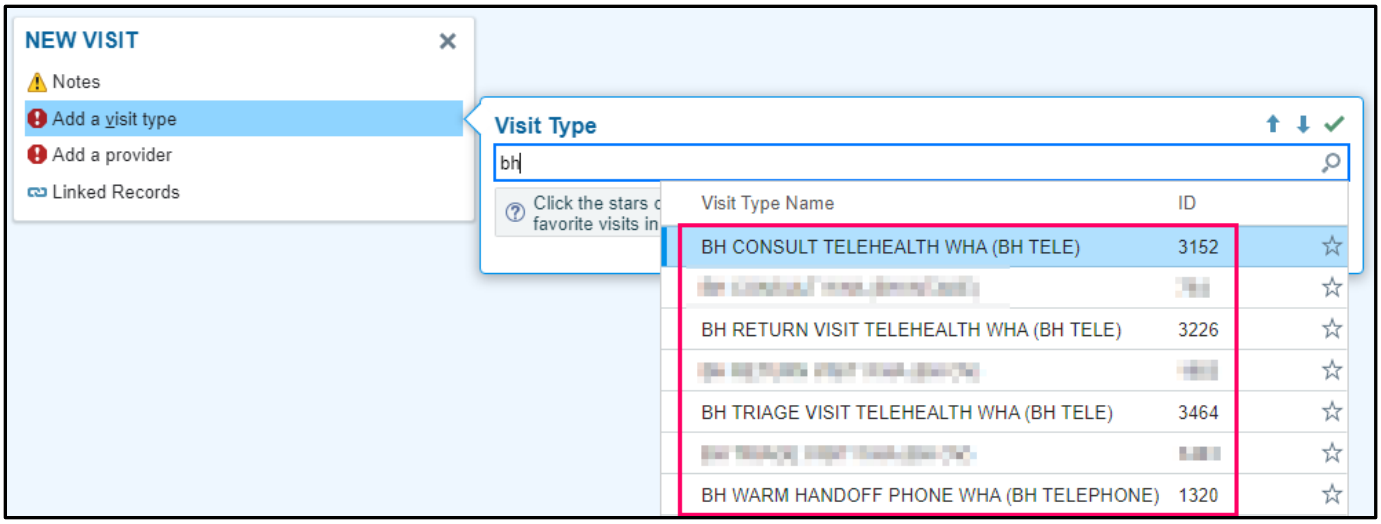
Scripting:

"We'll be sending you an important questionnaire through MyHealth, we ask that you please complete it prior to the day of your PP telehealth appointment. If you complete the questionnaire prior to your appointment you will not receive a call to roomed, but you may receive a call from a nurse to discuss your answers to the questionnaire. You will still receive a text from the provider to join your visit at your scheduled appointment time. If you are unable to complete this questionnaire prior to your appointment, then a clinical team member will be contacting you to go over the questionnaire prior to your appointment with the provider."

BH Telehealth Appointments

[Click this link to the full BH Appts and Workflows tip sheet]

There are four visit types for our Behavioral Health Telehealth appointments



These visit types will automatically default to 60 minutes *(minus the warm handoff phone which is 30 mins)*

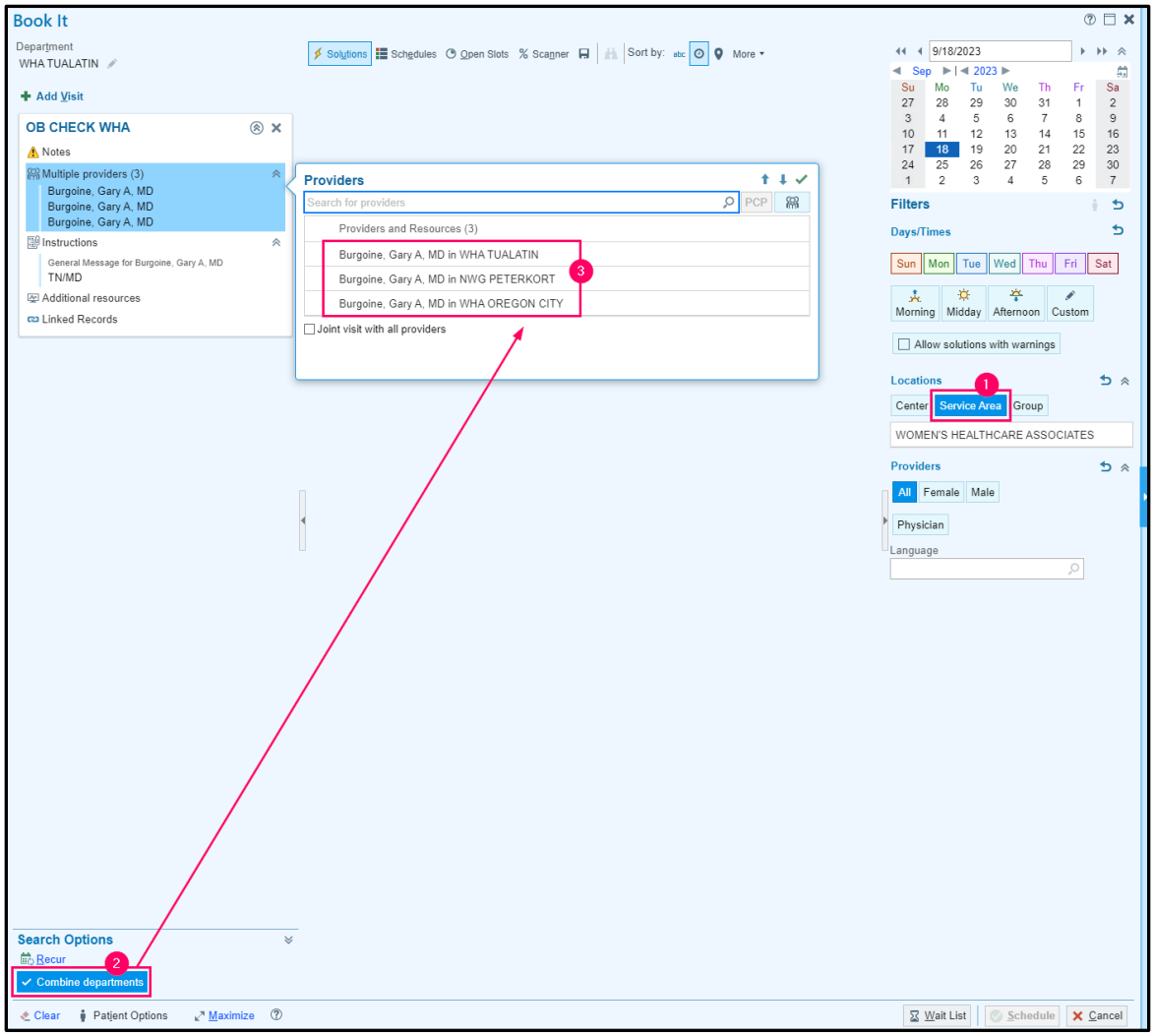
- **BH CONSULT TELEHEALTH WHA:** For patients who are new to BH care or have not been seen by BH in 6+ months *(can only be scheduled by the BH PSR)*
- **BH RETURN VISIT TELEHEALTH WHA:** For returning/follow up patients that have been seen by a BH provider within the past 6 months
- **BH TRIAGE VISIT TELEHEALTH WHA:** Only schedule when referral says Triage visit
- **BH WARM HANDOFF PHONE WHA:** Only used by BH Providers when making phone calls regarding a patient's WBA form answers

Note: If you receive a call from a BH patient and you are unsure of what to schedule or do not feel comfortable scheduling, you can to reach out to Bianca, or the BH PSR for assistance, or send a Staff Message to the Patient Services Behavioral Health pool and they can follow up with the patient in a timely manner.

Scheduling across WHA Departments

This workflow is important for those clinicians that work EH and Saturday shifts that will be listed under a different Department in EPIC.

To search for a provider across multiple sites, follow the steps below:



1. From the Book It screen, verify that Service Area is selected
2. Check the box **Combine Departments** to show all WHA Departments the Provider(s) are assigned to
3. All assigned Provider locations will now show up here

What to transfer to Advice over scheduling a Telehealth:

Reference the WHA Scheduling Timeframe Guide for what and when to call or transfer to the Advice nurses

If after reviewing that document you are still unsure, call the **Advice Priority line** in Five9

Telehealth Appointment Review:

After scheduling a Telehealth visit for a patient, you will see some patient and COVID related information added to the **Appointment Review** window under **Patient Instructions and Location Instructions.**

Make sure you read these Patient/Location Instructions to the patient after scheduling them !!



Scheduling New OB visits including initial Telehealth

Appt Notes for NOB visits:

OB Intake Telehealth (6-8 weeks) - VIDEO Visit: Pt aware of text link – LMP date - Ph# xxx-xxx-xxxx

OB Prim Consult Telehealth (8-10 weeks) - VIDEO Visit: Pt aware text link – LMP date - Ph# xxx-xxx-xxxx

US/OB Workup/Lab visits (12 weeks) –

- **Ultrasound** – LMP date
- **OB Workup** – LMP date
- **Lab** – *leave blank (no notes)*

See this entire workflow under Patient Services COVID workflows on Sharepoint here:

Scheduling New OB visits including initial Telehealth

Advice Nurses

Advice Scripting

Advice Scripting for Telehealth Visits

From what you describe, your need would be best met by a telehealth video appointment with a provider for assessment / evaluation. I have an opening _____ with _____; can I go ahead and schedule you for that? Great, and I do want to let you know this is a regular visit, meaning your insurance company will be billed. Depending on your benefits, you could be responsible for a copay or deductible and co-insurance requirements.

- **Possible Patient Follow-up Question:** Can you just help me? That's what I normally do when I call
  - **Answer:** Due to the pandemic, our advice team is smaller and focused on triaging patient needs when there are no provider appointments available. I think a telehealth appointment is the best way for you to get timely care right now

- **Possible Patient Follow-up Question:** Does the Telehealth visit cost the same?
  - **Answer:** Virtual visits are billed to your insurance company just like a regular office visit. Depending on your benefits, you may be responsible for a co-pay or deductible and coinsurance requirements

- **Possible Patient Follow-up Question:** Well, if I'm going to be billed, I'd rather be seen in person.
  - **Answer:** If you and your provider decide you need to be seen in person after the Telehealth visit, we will get you scheduled to come in

- Ask what is the best phone number to reach the patient on during the appointment time

- **Add** appropriate information to the appointment notes (see below):

If **Roxana (WHA's Internal Spanish Interp.)** is the interpreter for the appt, use these Appt Notes:

VIDEO Visit: Pt aware text link, Roxana to interpret-3rd party call them at 503-905-4355

Appointment types	Appt Notes
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• EST Gyn • NP Gyn • OB Intake • OB Prim Consult • OB Telehealth • Postpartum • BH	VIDEO Visit: Pt aware Text Link - Wks gestation, LMP or Visit reason - ph# 000-000-0000
Linguava Interpreter needed for any Telehealth *Note if it is a Phone Visit, add that to the beginning of the Appt Notes	Linguava needed, see telephone encounter - Visit reason Or Phone Visit, Linguava needed, see telephone encounter – Visit Reason

- After Scheduling, give these instructions to the patient:
  - *"At the time of your visit, you will receive a text message from your provider. All you have to do is click the link in the text message and you will be connected via video with your provider for your visit. A few things to remember: try to be in a quiet, private location with good cell service or reliable Wi-Fi. For best audio quality, we encourage you to use headphones. If you are not able to use headphones, avoid placing your phone on a surface that blocks the speaker/microphone."*

For instructions on scheduling and using interpreters vs. clinician interpreters see Patient Service section

Scheduling Same Day New Patient Telehealth Visits

New Patient Same Day Telehealth visits need to be scheduled at **least one hour** before the scheduled appointment time

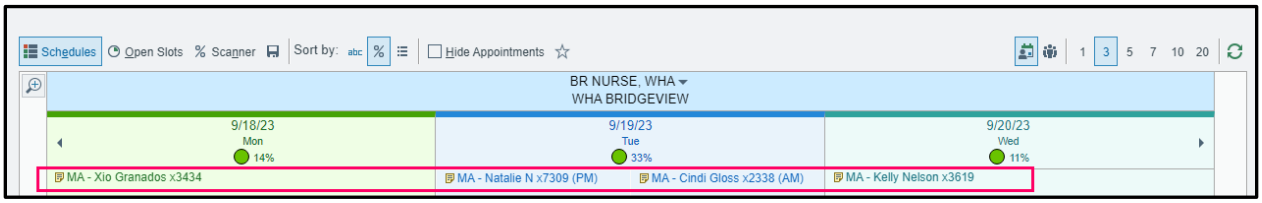
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Try to avoid scheduling new patients same day for Irregular bleeding, abnormal bleeding, or post-menopausal bleeding unless they are insistent on being seen today

Communication:

Let the patient know that they will receive a phone call from the clinical team member within the 2 hours prior to their visit to prep their chart and if they miss this call or do not complete their eCheck-in via Myhealth they will be rescheduled

Next, instant message in Teams the telehealth rooming MA notifying them of the add-on. The rooming MA's name can be found in EPIC at the top of the schedule view of the nurse schedule for each HUB (**BR Nurse** for all hubs)



Scheduling Telehealth visits that need an interpreter:

When a nurse is scheduling a patient *(who needs an interpreter)* for a Telehealth visit that is same day to 3 business days out, a **high priority Staff Message** will be sent to the corresponding Patient Services pool letting them know they need to schedule an interpreter and create a Telehealth Interpreter Encounter.

South Hub Telehealth visit = OC patient services pool

West Hub Telehealth visit = HB patient services pool

East Hub Telehealth visit = TA patient services pool

If the Telehealth appt that needs an interp. is scheduled **more** than 3 business days out, no Staff Message is needed as there is a report the PS team is running to catch all future appts.

The Nurse will provide a 1 hour lead time when scheduling these appointments, as this will provide adequate time for the Patient Services team to document the needed interpreter information in the patient's chart for the clinician.

If you need to schedule an appt that will start within 1 hour, you must call a Patient Services member and ask them to create this Telehealth Interpreter Encounter ASAP.

When a Patient Services team member receives a high priority Staff Message indicating a patient needs an interpreter for a same day/upcoming visit, they will document the needed interpreter information in the patient's chart for the clinician.

Medical Assistants/Rooming Staff

Telehealth Virtual Rooming Workflow

Overview: With MA/LPNs rooming patients via telehealth, they will follow standards in an effort to be efficient while still obtaining the required information for the clinician. These standards will apply to ALL clinicians and visits (excluding behavioral health).

FAQs:

How should I contact the provider if I need to alert them of something? Send them a Staff message, call or Teams message (site specific)

I wasn't able to get ahold of a patient prior to their appointment. What should I do? The goal is to make 3 call attempts to each patient and document all 3 call attempts in the same Telephone Encounter. Begin by starting a Telephone Encounter and route it to the clinician. This will allow PS staff to contact the correct clinical person if a patient calls back, and it will also let the clinician know that you were unable to contact the patient.

Scheduling Dots: Please use a **Black** dot on the left side of the schedule if you are unable to reach the patient prior to their appointment. Use a **Green** dot on the left side of the schedule if you successfully roomed the patient.

What does this  **icon on the schedule mean?** If there is a nurse visit with this icon  on the schedule, the roomer will need to ensure that they are checking in the nurse visit AND the provider visit. They will also need to ensure that they are doing their rooming/documentation from the provider schedule NOT the nurse visit schedule.

What about an interpreter patient? The patient services team is making telephone encounters for all interpreter patients that include the number for Linguava, the customer code, insurance and language.

What HPI smartphrases should I use? WHA has many standard phrases that should be utilized. If a patient has any of the below complaints, please use the corresponding smartphrase in the HPI, and answer the questions with the patient (could be provider or site specific).

What should I do if a Note template already exists? At times, clinicians may start writing notes for their Telehealth visits ahead of time. If a clinician has already started filling in a template and making some notes, do not delete their work. Instead, review the note and consider filling in any gaps or placeholders during the appointment with the patient as needed.

New Patient Telehealth Appts

New Patient **same day** Telehealth visits will be scheduled at **least one hour** before the scheduled appointment time. They will NOT be scheduled if pertinent records are not already in the patient's chart or are in Care Everywhere, unless the visit does not require records (BC consult, options counseling, hasn't been seen for this concern anywhere else, etc.)

All new patients being seen for a telehealth appointment are **required to complete their consent forms prior** to being seen. Just like we wouldn't have a new patient roomed in office until they sign their consents up front, this will be a requirement before their provider can see them on telehealth. We cannot bill their insurance if the consent forms are not signed prior to their visit.

How will patients sign consents if they aren't in office?

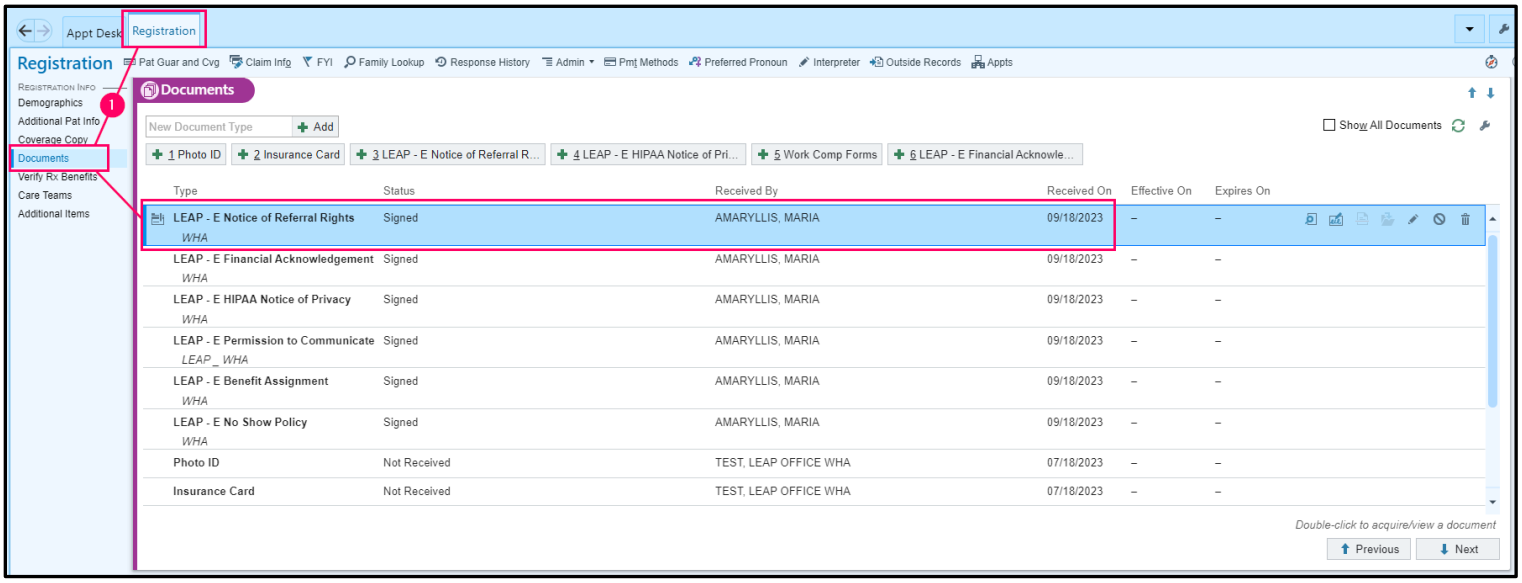
Answer: Through eCheck-in via Myhealth

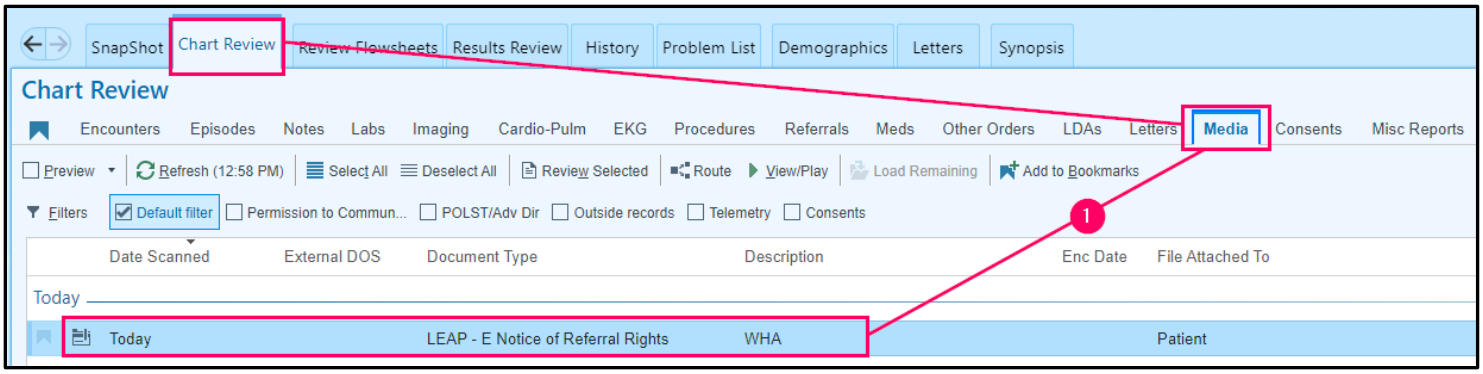
Any patient who is New to WHA, or has not been seen in 3+ years and is being seen for a telehealth visit will need to sign up for MyHealth and complete the eCheck-in process **If patient has not completed consents/signed up for MyHealth, follow the e-check in workflow here:**

See e-Check In/"Get Ready" Workflow for more details

The next section shows you how to locate the consent forms to verify they have been signed before rooming the patient.

Locating Completed Consent Forms





1. To view the signed consent forms:
    - From Registration, click on the **Documents** tab (top photo)
    - From the Chart, click on **Chart Review** then the **Media** tab (bottom photo)

Communication:

The scheduler (PS/Advice team) will let the patient know that they will receive a phone call from the clinical team member within the 2 hours prior to their visit to prep their chart and if they miss this call or do not sign their consent forms, they could be rescheduled depending on what visit is scheduled (Wellness and NP Telehealth visit).

If the Patient Services team adds a NP Telehealth visit same day, they should send an instant message in Teams to the telehealth rooming MA notifying them of the add-on. The rooming MA's name can be found in EPIC at the top of the schedule view of the nurse schedule for each HUB (BR Nurse for all Hubs). When the rooming MA receives this instant message from the scheduler, they need to reply to let them know they received it.

Last the rooming staff will need to call the patient and prep their chart within 2 hours of their scheduled appointment time.

The MA will call the patient prior to their appointment to "room" the patient.

MA Scripting: "Hello! I am *** and I will be gathering information for your provider prior to your appointment. Before we start, can you confirm your name, date of birth and address? Can you also confirm that you are currently in the state of Oregon and will be for your appointment with your provider?"

After completion of rooming:

- **MA to let patient know that they will receive a text message with a link. They will need to click the link and it will take the patient to the meeting**

Postpartum Visits

If there is a nurse visit with this icon  on the schedule, the roomer will need to ensure that they are checking in the nurse visit AND the provider visit. They will also need to ensure that they are doing their rooming/documentation from the provider schedule **NOT** the nurse visit schedule.

If there is a PP telehealth visit on the BR Nurse schedule, it means the patient was not able to complete their EPDS prior to their appointment time.

- Since patients may complete the questionnaire between the time of the reminder phone call & time you are scheduled to call them for rooming, check their chart to ensure they have not completed the EPDS prior to calling them

- MyHealth forms are currently only in English, so any of our patients needs interpreter services will remain on the BR Nurse schedule

Postpartum 3 weeks

- Verify Correct Patient
- Verify patient is and will be in Oregon for their appointment
- Arrive the patient on the schedule
- Pull in Visit Template if not already added
- Put in Chief Complaint
- Vitals-weight/LMP/BP if they have a cuff. Add note "BP obtained via phone with MA."
- Verify Allergies
- Verify Medications and Pharmacy
    - If patient isn't taking medication regularly – Mark not taking
    - If patient has discontinued the medication - Discontinue it in chart
- Verify and update past histories

- Add in orders for labs as needed
- Update Postpartum information in episode
- Complete the EPDS screening, documenting it in the HPI
- Fill out HPI with postpartum 3 smartphrase (.WHAHPIPOSTPARTUM3)
- Visit follow up

GYN Visits

If there is a nurse visit with this icon  on the schedule, the roomer will need to ensure that they are checking in the nurse visit AND the provider visit. They will also need to ensure that they are doing their rooming/documentation from the provider schedule NOT the nurse visit schedule.

NP GYN Visits:

- Verify Correct Patient
- Verify patient is and will be in Oregon for their appointment
- Verify e-Check has been completed or walkthrough with patient to sign all consent forms
- Arrive the patient on the schedule
- Pull in Visit Template if not already added
- Chief Complaint
- Vitals-weight/LMP/BP if they have a cuff and smoking history. Add note "BP obtained via phone with MA."
- Input menstrual history
- Verify Allergies
- Verify Medications, refills and Pharmacy
    - Tee up medication refills (if unsure, add a note to clinician)

    - If patient isn't taking medication regularly – Mark not taking
    - If patient has discontinued the medication, discontinue it in chart
- Fill out WBA (If WBA is unavailable, manually fill out PHQ-9)
- Input past obstetric history
- Input past medical history
- Input past surgical history
- Input family histories
- Add in orders for labs as needed
- Fill out HPI with history of problem. Use the WHA specific HPI smartphrases whenever possible. If problem doesn't have a specific HPI, gather data (i.e. when did problem start? Has anything helped or made it worse?)

***Saturday ONLY* EST GYN Problem Visits:**

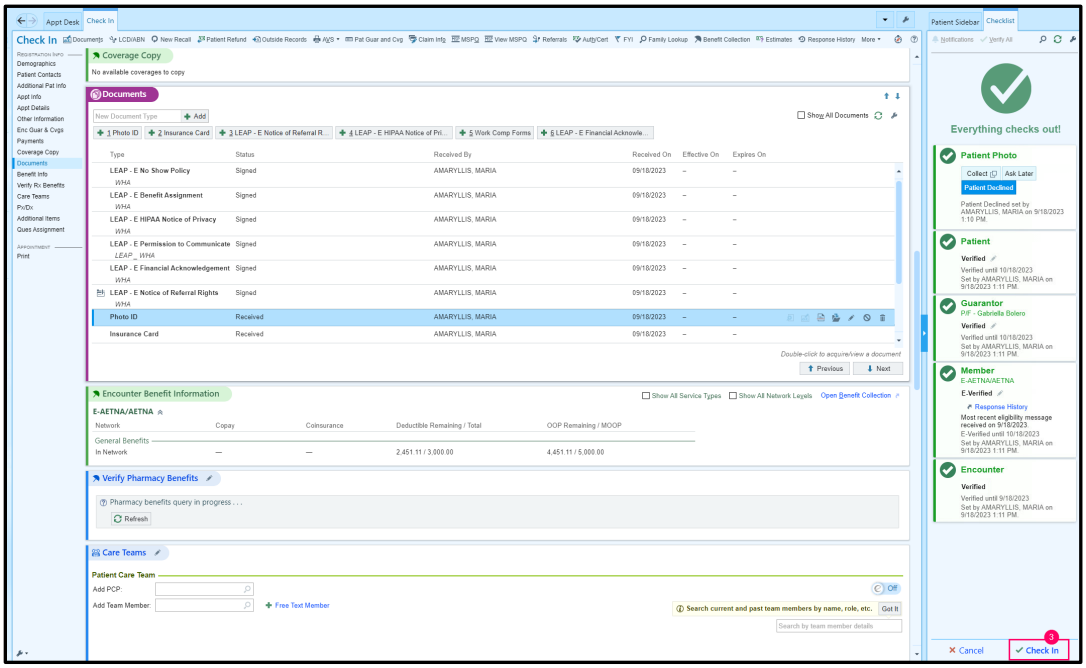
- Verify Correct Patient
- Verify patient is and will be in Oregon for their appointment
- Arrive the patient on the schedule
- Pull in Visit Template if not already added
- Chief Complaint
- Vitals-weight/LMP/BP if they have a cuff/menstrual status and smoking history. Add note "BP obtained via phone with MA."
- Input menstrual history only problems with bleeding
- Verify Allergies
- Verify Medications, refills and Pharmacy
    - Tee up medication refills (if unsure, add a note to clinician)
    - If patient isn't taking medication regularly – Mark not taking
    - If patient has discontinued the medication, discontinue it in chart
- Verify and update past obstetric history
- Verify and update past medical history

- Verify and update past surgical history
- Verify and update family histories
- Add in orders for labs as needed
- Fill out HPI with history of problem. Use the WHA specific HPI smartphrases whenever possible. If problem doesn't have a specific HPI, gather data (i.e. when did problem start? Has anything helped or made it worse?)

Checking in a Telehealth visit for Telehealth rooming

1. From your appointment list, click once on the patient
2. Click once on **Check in** on top tool bar

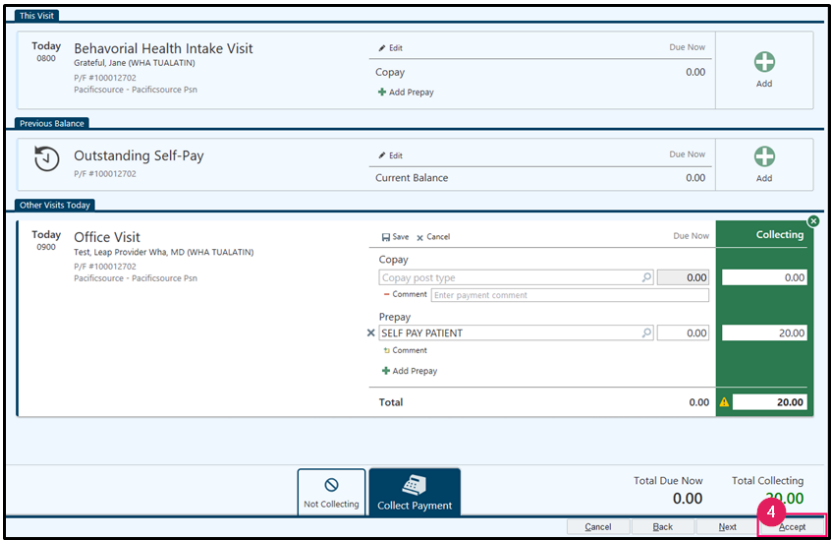


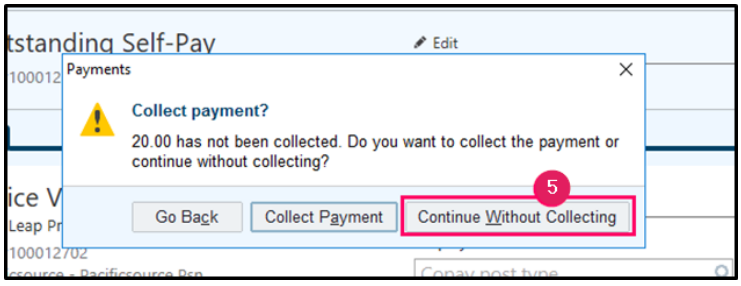


3. Once in the Registration click **Continue check in** in the lower right corner

4. Click **Accept** in the lower right corner

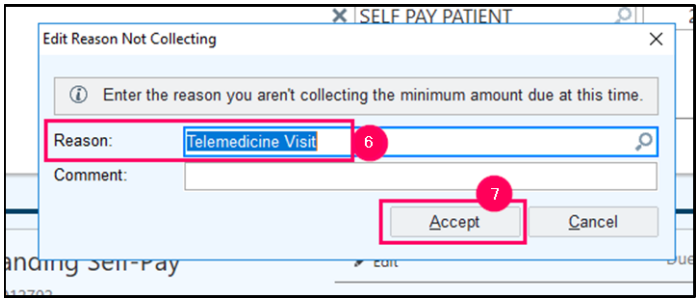
5. If the patient has a balance due at check in or a copy for the visit you will get a pop up. Click **Continue without collecting** to by pass





6. In the next pop up box you must enter a reason why you are not collecting payment. Choose **Telemedicine Visit**

7. Click **Accept**



8. Click **Accept** and continue with your normal Rooming process

